

400,000+ genetically-confirmable cases are born in India every year — yet rural diagnostic infrastructure barely exists, and up to 40% of cases receive misdirected initial treatment due to late detection.

THE PROBLEM

A child born with a treatable genetic disorder in rural India looks completely healthy at birth — and families remain entirely unaware. No visible signs. No reason to suspect anything. Until a missed milestone finally forces a hospital visit — by which point, the therapeutic window is already gone. This isn't negligence. It's structural. Rural health centers face an **absolute shortage of geneticists, counselors, and diagnostic equipment** — all concentrated exclusively in urban tier-1 facilities. In the absence of early screening, most conditions go undetected, up to 40% receive misdirected treatment, and what was manageable becomes permanent. Rural India isn't missing these children. It's missing the tools to find them in time. **GenSetu exists to change that.**

THE SOLUTION

GenSetu is a decentralized, three-pillar screening architecture deployable by an ASHA worker with nothing but a smartphone.

Pillar 1 — AI-Powered Computer Vision. A smartphone-native screening engine that performs instant phenotypic risk stratification at the point of care — zero specialist dependency, zero additional equipment.

Pillar 2 — Dried Blood Spot Logistics. Flagged cases trigger a single finger-prick sample, stable at room temperature — no cold chain, no expensive transport.

Pillar 3 — Automated Bio-Informatic Triage. Confirmed cases are instantly classified into two pathways: conditions manageable through physiotherapy, nutrition, or lifestyle intervention are routed directly to a care protocol; conditions requiring advanced therapy are auto-escalated — lab referrals initiated, government rare disease fund applications pre-filled with the diagnostic report attached. No paperwork. No navigation burden on the family.

WHY NOW, WHY US

Global leaders like Face2Gene serve urban clinicians but are trained predominantly on non-Indian faces, with documented gaps in accuracy. Lab networks like MedGenome have DBS infrastructure but no AI-driven front-end or rural reach. **No existing player connects rural screening, cold-chain-free logistics, and government-funding navigation into one pipeline** — that white space is GenSetu.

BUSINESS MODEL

A hybrid revenue model: a per-screening fee from state governments, integrated into existing RBSK/NHM budget lines, and a referral fee from diagnostic lab partners for every pre-qualified, high-probability DBS sample routed to them.

THE ASK - GenSetu is at prototype stage. We seek mentorship to stress-test our model, validate our clinical approach, and identify the right partnerships to move from prototype to a live field pilot.